

Between Negative Stigma (Cultural Deprivation) and Positive Stigma (Learning Disability): The Historical Development of Two Special Education Tracks

Ofer Katchergin

Published online: 12 October 2012
© Springer Science+Business Media New York 2012

Abstract This article posits an updated, broader perspective on the concept of learning disabilities (LDs) than that accepted in the local Israeli literature, revealing how it is immersed in class, ethnicity, and culture. This is shown through historical description, accreditation, and contrasting of the two special education discourses: the “cultural deprivation” discourse and the “LDs” discourse. There are three sections. Part One presents the theoretical, conceptual, and methodological background of the sociological and discursive debate about LDs. The social-constructivist model used in an analysis of the two categories is proposed as an alternative to the clinical–medical model. The definitions of LDs and cultural deprivation accepted in the Israeli discourse are presented in Part Two. The metamorphoses in the discourse about the category of LDs are uncovered through reference to their conceptual and historical antecedents. This part discusses the various understandings and constructions of learning difficulties. Part Three examines the textual representation of parents of children with disabilities in both cases, exploring the meanings of guilt, responsibility, and agency in each discourse. The conclusion clarifies the social and political significance of the distinct textual and rhetorical representations. It becomes evident that the discourse on LDs and the discourse on cultural deprivation are two special education tracks directed at different target audiences: the culturally enriched audience, well-off and educated on the one hand, and the Mizrahi audience of limited means and education on the other hand.

O. Katchergin (✉)
Oranim College, Kiryat Tivon, Israel
e-mail: oferkerenor@gmail.com

O. Katchergin
Levinsky College, Tel aviv, Israel

Keywords Medicalization · Psychopathologization · Learning disabilities · Cultural deprivation · Critical discourse analysis · Social construction

Part One

Introduction

During the sixth decade of the twentieth century, two clinical and educational categories regarding learning and social difficulties were formed in Israel. One category—that of cultural deprivation—has disappeared from the contemporary Israeli public and academic discourse. In contrast, the second category—that of learning disabilities (LDs)—has gathered ever greater and greater momentum and public exposure. The prestige of the first category has been waning due to the long series of social and professional criticisms that it was subjected to for many years. The prestige of the second category has been rising though and it is now enjoying an almost unanimous acceptance among academic researchers as well as educators and practitioners.

Both of those two historical categories purportedly delineated specific medicalized conditions that were very different from regular learning difficulties: cultural deprivation expressed learning difficulties and failure emanating from socio-economic and cultural sources, while LDs expressed learning difficulties and failure emanating from biological sources. Interestingly, to date, no academic efforts have been made in Israeli professional and educational literature to juxtapose those two clinical categories and scrutinize them reciprocally.

This juxtaposition seems necessary on the backdrop of the historical parallel evolution of the two special education discourses that made the existence of the two clinical categories possible. Over the years, the two discourses developed in parallel asymptotically establishing distinct territories of work and knowledge. Different disciplines, professionals, and practices developed in order to aid and treat the varying needs of the two types of population that were constituted as subjects of these two special education discourses. Points of contact between the two discourses were almost entirely absent. It is worth mentioning that even the critical sociological and educational literature about cultural deprivation ignored its historical and educational twin and the social implications of that simultaneous evolution.

The current article seeks to throw light on the aforementioned silence by juxtaposing the two special education discourses while emphasizing their central points of similarity and of divergence. The article delineates the two clinical twin representations that were constructed in both of the discourses while accentuating their dissimilarities. Among those, one can find divergent representations of the “self” (both of the disabled and of their parents) that were constructed in the framework of the two discourses, different expectations of the two populations and their families, different causal attributions of the two syndromes and different educational ideologies in which the two discourses were embedded. The current article also aims to clarify the importance of the two discourses’ parallel analysis by interpreting its findings in the framework of Israeli society’s hierarchical ethnic and

class characters (a social aspect that has been dominant since the country establishment¹).

The claim is made that an analytical juxtaposition of the two special education discourses helps to reveal the different assumptions which the educational experts held with regard to the two populations. It is claimed that, from their very beginning in the 1960s, the two discourses were directed at distinct social populations. Whereas the objects of the cultural deprivation discourse were the Mizrahim, lower class and less educated, the objects of the LD discourse were Ashkenazim, the established and educated.²

There are three sections. Part One presents the theoretical and the conceptual background of the discussion. It presents claims based theoretically on the intellectual fields of critical discourse analysis, narratology and sociology of knowledge. It also clarifies its methodological guidelines while delineating the textual corpus that serves as its data.

Part Two presents the definitions of LDs and of cultural deprivation accepted in the Israeli discourse as well as their historical transformations. The metamorphoses in the discourse in relation to the category of LDs are uncovered through reference to their conceptual and historical antecedents. This part reveals the two aforementioned discourses' divergent conceptions and constructions of learning difficulties: Negative stigmatization in the cultural deprivation case vis-à-vis Positive stigmatization in the case of LDs.

Part Three further exposes the differences between the two discourses by examining the textual representation of parents of children with disabilities and families in both cases, as well as the different expert expectations in the two cases. The distinct discursive positioning of the families in both cases (parental inclusion and participation in the LDs case vis-à-vis parental exclusion in the cultural deprivation case) uncovers important distinctions in the families' textual representations while exploring the meanings of guilt, responsibility, and agency in each discourse. The learning disabled parent's constructed profile is that of a good parent in potentiality: an imagined ideal parent helping his child to overcome the disability. The culturally deprived parent's constructed profile is that of a pathological case comprising a central component of the child's problem. The expert literature consistently blurs the learning disabled parent's ethnic and class characteristics while it contrarily highlights the culturally deprived parent's ethnic and class characteristics. Finally, the article explicates the social and

¹ Israel Jewish population was divided from the onset of the state into two distinct ethnic groups: Ashkenazi Jews and Mizrahi Jews. During the 1940s, 1950s, 1960s, and most of the 1970s, the political, economic, and cultural establishments in Israel were governed by Ashkenazi elites. The Mizrahi immigrant population suffered from practices of marginalization and exclusion within the Jewish population in Israel (even though the two populations were about the same size). Moreover, since 1969, the marginalized Mizrahi population outnumbered the Ashkenazi population. For relevant statistical and demographic data of the aforementioned decades, see Peres (1976). For concurrent data and analysis of ethnic relations in Israel, see Shitrit (2004).

² The term Mizrahi (pl. Mizrahim) is a common appellation in Israel for Jews of Middle-Eastern and North African (mostly Arab) origin. Ashkenazi (pl. Ashkenazim) commonly refers to Jews of Western (mostly European) origin. These terms are often used by Israelis in order to draw cultural, hierarchical distinctions between the two groups. Traditionally, the term Ashkenazim designated a better symbolic status, while the term Mizrahim had negative cultural connotations.

political implications of those distinct textual and rhetorical representations by highlighting their embeddedness in the Israeli ethnic and class social structure. The textual representations constructed a purportedly strong link between good parenthood and a well-off socio-economic class holding a distinct, significant cultural capital. In contrast, the texts constructed a strong link between problematic parenthood of limited means lacking in both economic and cultural capital. Thus, the LDs discourse (much like the cultural deprivation discourse) could have been used potentially from its very inception as a divergent special education track serving a distinct social population. The discourse on class and ethnic embeddedness has not been dealt with in the Israeli academic literature. Revealing the conceptual, historical, and cultural foundations of the current Israeli, LD discourse can eventually contribute to the development of a more theoretically complex discourse by exposing its cultural and political context as well as the social forces shaping it in the present and possibly in the future.

Methodology and Corpus Selection

The professional discourse is analyzed by means of discourse analysis and narrative analysis techniques. Using these, the article focuses on the different ways that enable the LD experts to enforce their version of social reality with the aid of various linguistic manipulations. In this article, discourse refers to routine mundane practices that produce, stabilize, and enforce social relations characterized by social control and hierarchical distinction between social groups (Thompson 1984). Narrative refers to routine interpretative devices through which people represent “reality,” “self,” and “disease” conceptions to themselves and to others (Riessman 2008). Furthermore, these processes are also constructed in written texts through the use of linguistic strategies of symbolic construction (Thompson 1984).

The daily social use of language produces and reproduces various meanings through its narrative components (Colyar and Holley 2010; Horsdal 2012; Riessman 2008; Toolan 2001). Claims that are made regarding the social reality are often perceived as self evident. This is an expression of the power of language to produce such self evidentiality (Fairclough 1992). For example, this is the position of the clinical explanations embedded in professional written texts serving as data in the current article.³

The current research is based on a rich textual corpus which includes two types of professional and academic literature: Israeli literature about cultural deprivation (*Teinut Tipuach* in Hebrew) and Israeli literature about LDs (*Lekuyot Lemida* in Hebrew). In both cases, the material selected for analysis included texts which were regarded as central (canonical) in their fields as well as other texts.⁴ The earliest of

³ The present article focuses on the claims made by clinical experts and by authoritative texts.

It should be mentioned that there is also a considerable amount of writing that tries to problematize clinical “objective” categories by bringing back to the discussion the voices of the labeled themselves. This writing stems from the academic field of “Disability Studies.” For major contributions to that field, see Linton (1998) and Oliver (1990). The voices of the labeled themselves (and their practices of resisting and deconstructing the authoritative texts) are the concern of a forthcoming article of mine.

⁴ Among these are texts which belong to the earliest publications about learning disabilities in Hebrew (even though they are not necessarily identified in the learning disabilities discourse as canonical. example of such a text: Roswell and Natchez 1969).

the analyzed texts were published during the sixth decade of the twentieth century (concomitant with the inception of the LD field in Israel). The selection of the LD literature and the delineation of the corpus for the purposes of the current research were made in two ways. First, with regard to LDs, a thorough archival scan was made in the various Israeli academic education libraries. Thereafter, all the texts which included LDs (as well as brain damage) as their subject were selected.⁵ Second, interviews were conducted with various didactic-diagnostics who were asked to state what, in their view, were the most important Israeli (canonical) texts in their field of expertise as well as what written texts had influenced them the most professionally. Following a triangulation of the interviewees' answers, additional specific texts were selected and further archival work was conducted in order to locate those texts whose authors were mentioned in the interviews as being central figures in the Israeli LD field.⁶ Regarding the cultural deprivation literature too, a thorough archival scan was conducted in the Israeli academic education libraries. Second, various education researchers were asked to state what, in their view, were the most important Israeli (canonical) texts in the cultural deprivation field. Various central texts were then selected. A special effort was made to locate those texts which were identified in the Israeli critical-pedagogical and sociological literature as written by central figures in the cultural deprivation field.⁷ The work was based on both primary expert literature and on secondary sources.

On the Social Construction of LDs and Cultural Deprivation

Various researchers, basing themselves on the social construction paradigm, claim that clinical knowledge (medical, psychological, and special-pedagogical) that is usually regarded as objective and neutral is, in fact, socially constructed (Bury 1986). In terms of this social model, the objects of study of the various clinical professions are not decontextualized neutral and objective objects. Rather they are objectivizations (Berger and Luckman 1967). That is to say that clinical entities are more similar to socially originated inventions than they are to decontextualized discoveries. Clinical entities are constructed within the framework of a medicalizing and psychopathologizing model by manipulating scientific ideologies, terminologies, and technologies. Nowadays, this model is producing an alarmingly increasing number of clinical categories characterized by distinct mental and physical qualities (Carter et al. 2012; Conrad 1992, 2006, 2007; Riska 2003).⁸ On the current state of the medicalization theory and on the different conceptualizations of medicalization within it, see Clarke et al. (2009, 2010). On the various meanings of the concept of medicalization and on its evolution, see Nye (2003). In developed countries, this

⁵ Brain damage was one of the most popular terms that predated the learning disabilities terminology in Israel.

⁶ Among the most prominent of these writers are Rivka Kidron, Shimeon Simpson, Shlomo Sharan, Yael Sharan, Ali Darin, and Rafael Belgor.

⁷ Among the most prominent of these writers are Carl Frankenstein, Gina Ortar, Hinda Eiger, and Moshe Smilansky.

⁸ The concept of medicalization was framed by Zola (Zola 1978) to signal extensions of medical jurisdiction, authority, and practice into increasingly broader areas of people's lives.

psychopathologizing model has turned out to be the main model through which people think, experience, and understand their problems, their lives and their world. Within this framework, complex social phenomena are reduced to clinical symptoms which purportedly testify to their originating disorders or diseases.

The psychological and medical model spreads with the help of various social carriers which daily testify to its findings in routine practices and articulations; expert professionals who serve as an army in the processes of locating, diagnosing, and caring; clinically labeled subjects; different organizations (bureaucratic institutions in which the processes of clinical construction are taking place, or NGOs with social agendas speaking on behalf of those labeled); and other social sites in which cultural representations and imageries of diseases, disorders, and problems are reproduced again and again thus legitimizing practices of medical supervision and control (Clarke et al. 2010; Moghimi 2012; Pulkingham and Fuller 2012).

The medical and psychological frameworks are being implemented in a growing number of spheres of human experience thereby constructing new clinical categories and professional communities which are made responsible for their management and interpretation (Clarke et al. 2010). This process is happening nowadays in the educational sphere in which one can discern a current process of medicalization of educational phenomena, a process in which clinical professionals adopt medical explanatory and therapeutic models applying them to social and educational matters (see, for example, Petrina 2006). The current article seeks to explicate the twin phenomena of LDs and cultural deprivation in light of the aforementioned medicalization process, by treating them both as particular cases of social construction.

Part Two

Between Negative Stigmatization and Positive Stigmatization: Different Representations of the “Self”

The definition of Brain Damage and LDs—delineating the problem and conceptualizing it in biological terms

In the research and professional literature, learning difficulties and problems have been conceptualized as part of the various medical, psychological, and education paradigms. The conceptualization of LDs, with all the terminology and theoretical models that explain them, only began to appear in the literature in Israel in the late 1960s. Prior to this, various other concepts were used by experts attempting to characterize and explain the quality of the learning difficulties and problems they came into contact with.

For example, in the initial decades of the twentieth century, there were widely used terms in the field of medical pedagogy in Israel such as: a-normal, organic, non-normal, educational difficulties, marginal, neurotic, problematic, lagging, to refer to children who displayed difficulties in learning various skills and abilities (Hamdani 2003; Simpson 1985; Razi 2006). In the 1950s, 1960s, and 1970s, the

field which had begun to be termed special education, identified children with difficulties as “having disabilities in brain function,” as “having brain function disturbances”, strephosymbolic, legastenic, hyperkinetic or exogenous. At times, they were described as having cerebral dysfunction, neuropsychiatric syndrome, brain defects, diffuse minimal brain defects, brain damage, disturbances in the central nervous system, hyperactivity syndrome, and so on (Eilon 1970; Birch 1970, Belgor 1977; Hess 1966; Arazi 1972). Another concept that was very popular during those years was cultural deprivation, a designation given mostly to children who belonged to families of Middle-Eastern origin of limited means.⁹ Contrary to the other concepts which were conceived as emanating from a biological–etiological infrastructure, the concept of cultural deprivation was believed to originate from an environmental etiological infrastructure (even though it still also had biological manifestations as a consequence of the environment characteristics).^{10,11,12}

A review of the books and articles in Hebrew on the subject revealed that, since the adoption of the term LD, the term LDs has become popular and has replaced the various aforementioned concepts (concepts which came to be regarded anachronistically as its predecessors). The historical terms were presented as erroneous, as theoretically, morally and ideologically charged, as damaging to the children, and as the subjects of many disputes. In contrast, the term LD and the range of terms of which it was composed, was identified as accurate, neutral, and objective, as not harmful and as one which enjoyed broad acceptance.¹³

Interestingly, the founding father of the Israeli LD field (as well as of the term LDs itself) was identified as the US professor, Samuel Kirk. Kirk defined LDs thus:

A learning disability refers to a retardation, disorder or delayed development in one or more of the processes of speech, language, reading, writing, arithmetic or other school subject resulting from a psychological handicap caused by a possible cerebral dysfunction and/or emotional or behavioral disturbances. It is not the result of mental retardation, sensory deprivation, or cultural and instructional factors.

⁹ The aforementioned Mizrahi Jews.

¹⁰ And see a specific reference to the cultural deprivation definition on page 11.

¹¹ Some of these terms were rapidly discarded (such as abnormal children or a-normal children). Others changed from time to time (the terms “children with battered brains” turned into concepts like “children with brain defects” and “children with brain damage”. From these, the terms “children with brain disturbances” and “children with minimal brain damage,” respectively, developed; and yet others have remained ingrained up to the present time, although their meanings and their related cognitive associations have altered. The latter include, for example, the term “dyslexic children.”

¹² The current study does not intend to examine and analyze the conceptual changes themselves (nor the historical circumstances that produced those changes). Such an examination and analysis require the implementation of precise archival-historical research which will require locating the connections between the changing concepts and their historical derivatives, and their professional and conceptual contexts, as well as the broader social and political contexts.

¹³ The learning disabilities field was established during the 1960s in the USA. In the year 1962, the US special education professor Samuel Kirk coined the term learning disabilities. Since then, this year has come to signify the starting point of that field in professional historiography.

Indeed, in Kirk's LD definition, emotional or behavioral disturbances are also mentioned as possible causes of LDs, but early in the field's inception, those potential causes were marginalized and were replaced by the cause which came to be known as neurological (cerebral dysfunction in Kirk's terms). Moreover, in later definitions of LD, emotional or behavioral causes were already defined as non-causal factors. Otherwise they were simply erased. No later than the late 1960s, Kirk's definition was replaced with new definitions. Those newer definitions are important for the purposes of the current article mainly in their explicit rejection of environmental, cultural, or economic factors as possible and relevant causes in the development of LDs. The neurological factor has thus become the only relevant etiological cause.¹⁴

The LD discourse was characterized from its very beginning by an empathizing and sympathizing portrayal of the human subjects who were constructed by it. Comprehensive descriptions were devoted to the psychological frustration, the difficulties, and even to the allegedly suffering of those labeled by it as they were portrayed as being in a constant struggle against their constitutional weaknesses as well as against the intolerant society. The learning disabled were also described as having intact motivation and intelligence. As a matter of fact, they were often described in the literature as being endowed with higher than average intelligence, capabilities, and potential. Especially interesting was the customary claim made by experts whereby failure in intelligence tests did not necessarily indicate the lack of intellectual capabilities or of cognitive potential on the part of the learning disabled child. Problematic manifestations of personality traits, familial relationships, or emotional difficulties, in cases in which they were found, were identified as consequences of the organic deficiency (and not as its generators). For example, in the framework of the LD discourse, LDs were a causal factor of problematic family relationships. The opposite process, in which problematic family relationships could have possibly been a causal factor of LDs, was rejected.

These characteristics of the LD discourse had a positive stigmatic influence. Therefore, it is not surprising that this discourse had a strong seductive potential from its inception. The problematic children who were constructed within the framework of the LD discourse did not resemble the previous multitude of brain damaged, brain handicapped, and brain defectives (the concepts that populated the expert literature prior to the introduction of the LD discourse). Likewise, the LDs children did not resemble the parallel discourse of the cultural deprivation children who were characterized as suffering from low intelligence, low motivation, lack of emotional maturity, and criminal-like lines of behavior.

Contrary to the negative stigmatization of the culturally deprived children, not only was the LD children's self not contaminated, but also, more often than not, these children's self came to be marked by augmentative qualifiers.¹⁵ In this manner, those marked as LD have come to be perceived more and more as intelligent and as motivational. As a consequence, previous medical and

¹⁴ Extensive reference to the developments and changes in the historical definitions of learning disabilities can be found in Katchergin (2009).

¹⁵ On identity contamination in cases which involve the imprinting of a stigma, see Goffman (1983).

pedagogical discourses (including the parallel cultural deprivation discourse) were turned upside down: Academic difficulties that were previously identified with mental retardation, with intellectual mediocrity, with laziness, or with moral weakness, have begun to be perceived as indicative of exactly the opposite traits!

The new LDs' discourse had many advantages. The self of the children who had academic difficulties was no longer contaminated, since that difficulty was delimited to the educational domain alone within its framework. The new discourse enabled the prevention of the neurological susceptibility to cross-over into other dimensions of the self-concept (such as personality, cognition, psyche, etc.). Thus, the children under discussion could become marked as having a coherent, integrative, and healthy self. Their cognitive, mental, and psychological identity remained without a blot on it, their identification as disabled (biologically originated) notwithstanding. Moreover, the neurological susceptibility discussed was conceptualized in an a-deterministic way, a conceptualization that facilitated the reception of the new discourse among groups of worried parents. That is, even though the susceptibility was identified as permanent by the expert diagnosticians, they still considered it repairable and reversible for their part. To a large extent, the LDs label had a unique role—it was a kind of positive clinical stigma^{16,17} that was in stark contrast to alternative clinical pathologizing categories that largely had (and some of them still have) a role of negative stigmatic labels.

The Definition of Cultural Deprivation—Delineating the Problem and Conceptualizing it in Environmental Terms

In the past, the term, cultural deprivation (as well as its equivalents, reading deprived and fostering required) was used to define students who failed in their studies for reasons that were identified as socio-economic and cultural. These students were perceived as requiring educational fostering; the term cultural deprivation was coined in the United States after the Second World War and gave expression to a perception that emphasized society's responsibility to advance neglected groups and to integrate them into the education system, society, and culture (Arieli et al. 1997). The reasons for cultural deprivation were attributed to education methods in the family, which were not appropriate for the Western middle class, its mindset, its language, and the school culture in these societies. A translation of these perceptions into the Israeli context was given major expression in the works of researchers and educators such as Frankenstein (1970), who claimed

¹⁶ The traditional literature on stigma focuses on identifying factors contributing to the harmful impact of stigmas on the lives of stigmatized individuals and social groups. In the current article, the term positive stigma is suggested for social processes in which medicalized labels do not devalue or discredit those labeled by them and do not spoil their normal identity. On the contrary, in this special case, the medicalized labels can rather empower the stigmatized.

¹⁷ It is interesting to note that a literal English translation of the Israeli term for culturally deprived (teunei tipuach) is “in need of nurturing.” This means that in Hebrew the term blurs the contextual elements (cultural and social) linked to it even more than the English term. Nonetheless, in this article, I chose to use the term culturally deprived which is more widespread in English. As for the Israeli educational notion of nurture (tipuach) and its local version of culturally deprived (teunei tipuach), see Mizrachi (2004).

that the thinking potential of these children was harmed because they were not exposed to sufficient stimuli at critical stages of their cognitive development.

In the early 1960s, the Israeli education system started to grapple with the problem of the culturally deprived by providing additional funding, like teaching hours, small classes, teaching aids and special programs, and teaching methods for schools that were defined as culturally deprived. It is interesting to note that, concurrently, similar steps were taken with regard to children identified as having brain damage or were “organic” (later termed learning disabled).

Indeed, in the 1990s, the use of the term culturally deprived in Israel was abandoned completely (Arieli et al. 1997). “It was found that it labels population groups on the basis of their ethnic origin, reinforces stigmas, and reduces their chances of extricating themselves from the situation.” The use of this term is no longer fashionable; however, it is apparent that the use of its historic twin—LDs—has become more and more widespread. The term LDs has established itself conceptually and practically in educational institutions and places which were previously occupied by other logical discourses.¹⁸

The cultural deprivation discourse was characterized by dis-empathizing and stigmatic portrayal of the human subjects who were constructed by it. The problematic children who were constructed within its framework were conceived as intellectually, emotionally, and morally damaged. Contrary to the case of the learning disabled children, the self-identity of the culturally deprived was cognitively, mentally, and psychically contaminated. Moreover, as opposed to the learning disabled children’s a-deterministic portrayal, the physical susceptibility of culturally deprived children was portrayed mostly in deterministic terms. The origin of the susceptibility was indeed conceived as environmental in nature, but still its physical manifestations were regarded as completely irreparable. To a large extent, the cultural deprivation label was used as a negative stigmatization (similar to many other clinical categories mentioned in the medicalization literature).

These two distinct channels of educational discourse and policy (that of cultural deprivation and that of LDs) developed concurrently in Israel. Therefore, it is surprising that the two special education discourses developed almost without any explicit mutual references. This is especially surprising when one takes into account that both focused on young and so-called problematic populations of the education system at the time. Even more notable is this mutual disregard given their common interest in identifying academic–educational problems, in locating their origins, and in developing pedagogical, didactic, diagnostic, and therapeutic intervention methods for the purpose of coping with them.

The parallel progress of the two discourses was also expressed in the creation of an apparent division of labor between experts in education and psychology who focused their attention on the culturally deprived and those who concentrated on the population of the brain damaged (at a later stage called the learning disabled). Both sets drew upon psychological and educational models and theories; both

¹⁸ It is possible that the growing popularity of the learning disabilities discourse played a role in the gradual elimination of the term cultural deprivation. The conjecture by which the learning disabilities discourse swallowed the cultural deprivation discourse which was held to be “problematic” and was extensively criticized should be investigated but this is beyond the scope of the current article.

formulated the educational problematics, often using the very same terms; both developed various means of identifying and intervening for the very same educational needs; and both wrote and published their studies in the same journals. So, this makes it more remarkable that there were almost no explicit references and mutual interchanges between the two discourses. And this does not include any consideration about the essence, the need for, and the sources of distinction between the two types of population—a conceptual and institutional distinction that was dominant and doxic as long as the term culturally deprived was in use.

Juxtaposing the Two Syndromes

Despite the differences in emphasis in the various definitions of cultural deprivation, several components have become accepted by the scholarly community and educators as basic to the definition of this educational problem. Darin (1984), for example, noted that “there are various definitions of the syndrome of cultural deprivation, but all of them have at least three fixed characteristics: low scholastic achievements, low learning ability, and lack of motivation for learning.” To these, he added that “in the accepted official definition in the United States, these components were supplemented by low intelligence quotient, and belonging to a low socio-economic stratum.”¹⁹ As early as 1966, Robinson determined that “reading deprivation” (a term that served in combination with cultural deprivation) has its source in a failed cultural environment together with low intelligence. He also declared that an objective measure of this was the parameter of father’s occupation (Darin 1984).

Sachs (1971) identified cultural deprivation with “environmentally instigated retardation” which he termed “pseudo-retardation” and linked the retardation to “cultural inadequacy, poverty, and primitiveness.” Daniel (1971) expanded on this and explained that “all the [culturally deprived] children, other than a few exceptions, belong to a low cultural stratum; their parents came from weakened Levantine countries, and, even there, belonged to a weakened socio-cultural stratum.” He, too, comes to the conclusion that the I.Q. of the culturally deprived is low “because of a cumulative deficit.”

And so it came about that the subjects of the conceptualization of cultural deprivation were identified and established in the framework of this discourse as unqualified cognitively–intellectually and motivationally–morally at one and the same time. A distorted mirror image of the organically damaged (the predecessor of

¹⁹ Darin was among the pioneers of learning disabilities research in Israel. In his writings, he gave only a marginal place to the subject of cultural deprivation. These remarks of his regarding cultural deprivation are taken from a subchapter devoted to “the development and education of children who are cognitively retarded.” A subparagraph in this subchapter is entitled “the culturally deprived child.” This almost anecdotal reference, the laconic definition, the identification of cultural deprivation with a “low mental ability” (both in its formal definition and equally by placing the discussion about cultural deprivation in a paragraph focusing on mental retardation)—all these stand in stark contrast to Darin’s lengthy writings about learning disabilities in the framework of which he emphasized, among other things, the “high mental ability” of the “learning disabled,” their abundance of motivation, and the great importance of developing a “sensitive attitude” to their distress.

the disability discourse) was attributed categorically to culturally deprived children.²⁰ It must be remembered that in the formative period of this discourse on LDs, the dominant opinion in this field determined that the children with organic issues have standard intelligence and standard motivation.

Etiological Differences

In contrast to the endogenous delineation of LDs, from the outset, the phenomenon of cultural deprivation was delineated as exogenous. For example, Stahl (1977) stated that “The term ‘culturally deprived students’ refers to those children with difficulty in their school studies for environmental reasons.” Various environmental models were developed which identified the causes of cultural deprivation with cultural, socio-economic, or family factors.²¹ The various models were trying to create a link between the problematic environmental conditions in which the children were raised and the personality and behavioral qualities which developed in them as a result of these conditions. These conditions were considered to be an expression of backwardness both in preserving this backwardness and as its outcome, and as ones that justified the need for educational projects for intervention and fostering.

The various written works present an index of characteristics attributed to the culturally deprived characteristics that were perceived as deriving directly from the backward lifestyle in which they were formed. Thus, it is possible to outline the profile of the culturally deprived self—that self that was shaped by the expert discourse on it. A central characteristic attributed to the culturally deprived was their lack of motivation to learn and to progress.²²

The experts made extensive use of concepts like lack of motivation, absence of mobility, weak desire for achievement, dependence on external authority, and lack of internal driving force to characterize the main aspects of cultural deprivation and also to provide an explanation for the long-term survival of the weaknesses of these students in the various educational frameworks. The weakness of the Mizrahi culture and the weakness of character of the culturally deprived student were melded together in the professional literature:

“Many anthropological studies have shown that cultural style plays a decisive part in the creation of the motivational structure of personality in earliest childhood” declared Emerant (1968), accentuating that “learning disruptions in the field of motivation are common on the backdrop of the crisis of moving from one culture to another.” “Our assumption is that this student will display little motivation to study, because those causes of motivation that are linked to studying are activated in his personality only in a mediocre or weak manner,” emphasized

²⁰ See, for example, Eiger and Kunik (1977), Frankenstein (1970), and Shmilowitz (1984b).

²¹ See, for example, Doron (1970), Smilansky (1978), Adiel (1969), Stahl (1977), and Shmilowitz (1984b).

²² See, for example, Eiger (1975) regarding the “lack of motivation” as a characteristic of culturally deprived children.

Adar (1968) in discussing the low desire for achievement of culturally deprived children: “The culturally deprived student displays lack of interest in his studies, in books, and in other intellectual pursuits. Even when the teacher manages to arouse his attention to what is happening in the classroom, he remains passive and lacking in enthusiasm. Due to weak motivation and deficient work habits, he is not able to comply with the tasks assigned to him and he does his homework in a non-standard manner,” stated Minkowitz in the preface to his book, “The Culturally Deprived Child” (1969).

Another important characteristic attributed to the culturally deprived is the so-called thinking deficiencies from which they were considered to be suffering, deficiencies which added explanatory reinforcement for the low achievements of the culturally deprived in the various educational frameworks. Terms such as damaged intelligence, deficient thinking, mentally immature, oriented toward the concrete, and low intellectual capacity were widespread in the expert descriptions of culturally deprived students.²³ These negative portrayals of the mental capacity of culturally deprived students were in common use by the expert community. These were the positions of Carl Frankenstein, educator and theoretician, one of the founders of the School of Education at the Hebrew University, and of Hinda Eiger, his student, who represented the spirit of the way in which cultural deprivation was treated in the professional educational literature.

Cognitive images of damaged intelligence were bound up with cultural images of backwardness and Mizrahi “eastern irrationality.” In the opinion of expert educators, it was only natural that the so-called irrational Levantine culture would give rise, in the field of education, to a damaged cognitive structure characteristic of irrational thinking whose family sources were based on a backward culture. Cultural deprivation and its educational cognates combined with other social domains in which the experts made an effort to create a hierarchical distinction between problematic cultural expressions of the backward Levant and the standard social patterns of the modern West.²⁴ Moreover, cognitive imagery about intellectual weakness was also integrated into professional perceptions and positions about poverty and socio-economic distress that were identified with the creators of thinking deprivation.

To top this off, additional major characteristics attributed to culturally deprived students related to the emotional and behavioral aspects of their personality. These students were identified as emotionally immature on the one hand and as displaying criminal behavior on the other hand.²⁵ The culturally deprived student was said to “be unable to withstand temptations and to delay gratification of disruptive needs” and that he has “a special negative attitude [to studies]” (Minkowitz 1969), that he is egocentric, impulsive, and emotional (Stahl 1977), that he expresses no interest in

²³ See, for example, Ortar (1966), Eiger (1975); Minkowitz (1969), Frankenstein (1970, 1981), Shalgi (1987), and Shmilowitz (1984b).

²⁴ See Razi (2006) about the task of drawing distinctions during the 1930s and 1940s in the educational field in Israel.

²⁵ On the criminalization of culturally deprived children through their teachers’ perceptions of them as close to populations of young criminals and as predisposed to deviant behavior, see Chasin (1975).

the act of learning for its own sake, since he has an instrumental approach to learning (Nissan 1973), that “strong emotions interfere with his thinking” and thereby blur the boundaries between them (Shmilowitz 1984a), and that he is cunning (Stahl 1977), and has tendencies toward criminal behavior (Minkowitz 1969). These qualities, too, were understood as stemming from the cultural and socio-economic context whose expression was interpreted as an outcome of the negative influence of poverty, neglect, and a culture that was not modern.

In stark opposition to this, in the LD discourse, there is a complete disregard of the cultural and socio-economic contexts as a relevant component in the conceptualization or the construction of the disabilities.

So, under the aegis of the burgeoning discourse on cultural deprivation, its expert proponents developed a kind of index of negative characteristics which related to the various aspects of the culturally deprived student’s personality. With regard to each aspect, ranges of personality weaknesses and defects were identified which the culturally deprived allegedly suffered from. The various qualities which composed the imaginary profile of the culturally deprived together delineated a weak self with defective functioning (Ben-Eliyahu 1976). The defects which characterized these qualities were together identified as symptoms of a defective self as noted by Frankenstein (1969).²⁶

In this way, the culturally deprived discourse produced a syndrome whose environmental (cultural and socio-economic) roots are expressed in the personality structure of those afflicted with it.²⁷

Summarizing this part, the current article seeks to point out the fact that in parallel to the drafting of the aforementioned cultural deprivation profile, there was a simultaneous drafting of a clinical profile that eventually came to be known as learning disabled. The representations of the “self” of both of these special education subjects were produced as distinct and different. Whereas the culturally deprived profile revolved around stigmatic characteristics interpreted connotatively as negative along with a disempathic description of its subjects, the learning disabled profile revolved, from the beginning, around largely similar characteristics interpreted this time connotatively as positive stigma.²⁸

²⁶ See also Minkowitz (1969) regarding this defective structure.

²⁷ More on the political role of psychological sciences in Israel in producing overt links between the “cultural backwardness” and “psychological impairment” of the eastern Mizrahi Jew, see Kashti (2009), Mizrahi (2004), Swirski (1990), Zalashik (2008).

²⁸ Former Israeli authors have already mentioned that the psychological models imposed upon the Mizrahi population in Israel replicated the cultural model of the normal and standard “Northern European Adult Protestant Man,” which was portrayed by Gaines (Gaines 1992), thus serving as this model deviant negative mirror image. Following Gaines’s analysis, I further argue that in Israel the psychological standard was based on the image of the “normative” Ashkenazi profile. Interestingly though the same “problematic” symptoms when manifested in Ashkenazi learning disabled children were identified as mere statistical variations of the normative cultural model and not as qualitatively different from it).

Part Three: Between Parents and Experts

The Importance of Sensitivity to Parents

A repeated motif in the LDs discourse deals with the great sensitivity that must be displayed in dealings with the parents of children with difficulties. The various scholars emphasized over and over the empathy they had for the parents' concerns. They also repeatedly instructed future practitioners to be sensitive to these worries. The explicit statements about the importance of sensitivity were supported by a series of textual mechanisms whose intertwining created a conventional rhetoric of empathization with parents, their children, and the suffering of both. Rhetorical mechanisms of empathization were directed at minimizing negative connotations, which were usually identified with disabilities in the structure and function of the brain.²⁹

Another central element which gave expression to the sensitivity and the identification of the experts with the parents of children with difficulties can be noted in the demoralization of learning problems and difficulties in the discourse on LDs. That is to say that in the framework of the new discourse, work was done on the negation and intensive conversion of moral and normative idioms that at times were identified as factors relevant to the understanding of learning difficulties in children. Over and over again, writers emphasized that children with difficulties should not be related to as having tendencies of lack of motivation, lack of curiosity or laziness. Demoralization was also given expression in relation to the role of parents and the family. They were no longer to be considered guilty or responsible for causing their child's difficulties.³⁰

In contrast to pedagogical-moralistic theories, the first experts on disabilities in Israel claimed that the parents should be helped to rid themselves of possible guilt feelings by uncovering the "real" reasons (that is to say, the biological ones) for the disabilities. The development of the disability discourse was characterized by the disappearance of moral indicators for disabilities; a disappearance that may have eased the espousal of the discourse for many groups of parents.³¹

The Image of the Good Parent: The Adequate and Helper Parent

The LDs literature addressed the parents as if they were "every" parent, stripping them of any particular markings of social and group affiliations. Nonetheless, the

²⁹ A detailed description of the various empathization mechanisms intended to minimize the negative connotations that were identified with dysfunctions of the brain structure and functions in the learning disabilities discourse is forthcoming in an article of mine.

³⁰ As opposed to psychological and psychiatric moral theories that characteristically blamed parental dysfunction as a central factor in the formation of disabilities. Theories like this include Bettelheim's "cold mother" theory (1967) and Ling's schizophrenogenic family (see Hartwell 1996; Singh 2004).

³¹ This does not mean that the discourse of disabilities lacked moral aspects, but that these began to take on a different quality. An expression of this can be found in the model of the ideal parent and in the creation of a system of norms obligating the parent of a learning disabled child. For example, the good parent was identified as non-judgmental, inclusive, helping his children with their work, involved in the educational activities of his children, and advancing them by means of private lessons, au fait with the disabilities discourse, and like a student in keeping up-to-date with expert knowledge.

texts contained a plethora of references, attitudes, comments, and remarks which hinted at the real parent audience which has been identified as relevant to the disability discourse: “The parents are required to devote energy and time to provide many motivational experiences to their children. Due to defective concentration and listening, the child will not persevere in these activities alone, and he needs an adult to encourage him and help him to persevere with his play. Only the parents can take the child to the park, to the beach, on trips, and thereby give him the required trust,” stated Sharan and Sharan (1969), and other experts reiterated the need to devote quality time to the children and how this contributed to their development. The diverse recommendations hint at the parent population likely to be characterized by certain entertainment and enrichment habits. Furthermore, this audience must be endowed with the resources of free time (as well as economic resources) such that the recommendations would be meaningful and implementable for them.

The parents were perceived as those that can and are expected to contribute their time to the act of teaching itself and to developing the children’s basic skills: “The parents must read age-appropriate books to their children from a very early age and keep them interested. This activity should be part of everyday life and should be pleasurable for both sides” (Darin 1984).³² They should lend a hand and help their children in preparing them for exams or in guidance and practical assistance (Segal 1969). The home environment constitutes a complementary, essential, and integral part of school instruction. Parents, identified as “an influential factor in the progress of the student”, are required to select reading material or, at least, to help with their reading, for the children’s independent work – materials like “articles from journals, short stories, novels, and imaginative stories.”

In order for parents to help their children scholastically in the best possible way, they were certainly expected to keep up-to-date with professional literature published in the field of disabilities, and in general to insure that they were aware of the knowledge building up in the field. Segal (1969) had already stressed the importance of “reading books, talking with other parents, participating in lectures and obtaining professional guidance” as essential channels for parents to “acquire sufficient understanding and knowledge to give adequate help to the child.” Many authors even add references to recommended literature and appendices containing lists of books for the parents to consult.

The parents were required to help their children not only in the educational-learning sense but also in emotional and social senses. That is to say that they were tasked not only with learning and teaching in the academic sense but also with studying the emotional, mental, behavioral, and social implications of their children’s difficulties, so that, as parents, it would be easier for them to understand, to include, and to accept them. The many expectations and enormous hopes that the experts assigned to the parents were accompanied by demands that they consult with them frequently and that they should develop concepts about proper parenting. Concurrently, normative claims began to surface from the parents, which addressed their needs and even their obligation to develop skills in these fields (academic, emotional, and social) so as to cope effectively with their learning disabled children.

³² See recommendations of this type in Segal (1969).

For example, Margalit (1984) made it obligatory to develop training programs for parents saying that “the change required of parents is a basic and protracted process, which can be seen as the development of a new skill – the skill of effective parenting.”

So the image of the imagined parent was of a parent with intellectual, motivational, emotional and social talent, able and encouraged (and more often obligated) to learn the professional jargon and to be aided by it, although this was modified by the fact that he must receive close guidance and assistance from a professional in the field. In this way, the experts encouraged the inclusion and integration of parents into the disability discourse while insuring themselves a position of superiority in it by creating an obvious dependency on them on the part of the parents who desire to update themselves frequently with the burgeoning knowledge.

Parent–Expert Cooperation

The authors’ recommendations did not only relate to the ways in which the parents ought to spend their free time with their children, whether it concerned their methods of entertainment or their teaching methods or their emotional and social formation within the home. The researchers also devoted much space in their recommendations to the ways parents should maintain communication with various educational entities on a regular basis: “The mutual relations between the home and the school are important for the progress of the child. The teacher and the parents must work in coordination so as to provide the child with a structured world in which there are reciprocal relations between the classroom and extramural activities... The parents of a brain-damaged child will often feel better if they feel as though they are participating in the child’s education and training” (Simpson 1974).

The parents were identified as potential partners in the act of education and treatment (Darin et al. 1976), not only as individuals in relation to the education system but also as a consolidated group (Segal 1969). In this way, various researchers promoted the organization of groups of parents for emotional and educational purposes at one and the same time. The parents’ cooperation with systemic entities is perceived not only as a contributory factor in remedial teaching but also as a central part of the success of the treatment.

The expectations and the recommendations of the experts that the parents should become involved in the educational effort carried out at the school, is not surprising given the identification of these parents as erudite, and as people familiar with or likely to become familiar with (even if only in potential) the professional jargon of the experts and with the scientific-educational knowledge of the time.

Furthermore, many experts explicitly position the parents as their partners in real understanding of the children’s difficulties, and make themselves the allies of the parents in their efforts to cope in the face of the refusal of the teachers and the education system in general to acknowledge that LDs are the explanation for the difficulties.

The Good Parent as a Source of Finance

The expert discourse assigns the good parent an additional important function—ongoing and long-term financing of the disabled child's treatment. Most of the authors have repeatedly stressed the great importance of providing external teaching assistance—that is assistance from private teachers and therapists who are not part of the school system. This function has been repeated over and over from the inception of the field of disabilities in Israel and is expressed in writing thus: “Parents of learning disabled children are supposed to get in touch with remedial teachers, teacher of extra lessons, counselors, doctors or nurses, and para-medical therapists (speech therapists, occupational therapists, etc.)” (Segal 1969).

The works by Rivka Kidron, one of the leading figures who took it upon themselves to develop awareness and policy in Israel for the field of LDs through advancing methods of remedial teaching, show the great importance the expert discourse attaches to individual education: “Remedial teaching means teaching aimed directly at the needs of the learner who does not manage to function as expected of him in terms of his mental level and his scholastic achievements in the classroom” (Kidron 1985).

For this reason, the expert discourse outlined the figure of the ideal parent: an involved parent who devotes time and energy to the educational act, while at the same time being aware of the importance of demarcating this involvement and delegating the management and organization of the child with difficulties to the expert. Also, the parent is expected to devote his economic capital to the educational effort on a regular basis. Funding private lessons for the child with difficulties is interpreted as part of the therapeutic act itself as it indicates sound parenting in that it is identified as an expression of worthy parental qualities like awareness of the problem and concern for the child.³³

Guilt and Responsibility

Since the parents are perceived as central factors with responsibility for the character of the disabilities, they are also identified with the theme and messages of guilt. Parental guilt has complex and paradoxical manifestations in the disabilities discourse. This is because, on the face of things, this discourse, which bases itself on pathological assertions about objective brain science, makes old-fashioned theoretical, psychological, and moralistic models redundant—models which found indications of pathology in children with non-normative parental conduct.³⁴ And so, despite all this, the figure of the good parent emerges from the expert literature as a figure of an active social agent that is commanded by means of the rhetoric of responsibility and parental concern to keep track of the proper development of his child and to really save him, if necessary.

³³ See, for example, recommendations for professional reading material together with references to recommended texts for parents in Sharan and Sharan (1969).

³⁴ More information about this can be found in Katchergin (2009, pp. 45–49) in the discussion about remedial teaching, scholastic strategies, and economic resources.

Despite the doxic biological conceptualization of LDs (as nervous disorders which have nothing to do with defective education or inappropriate environmental conditions), the written works of the experts have created the moralistic imperative (usually delicately formulated) whereby parents must take care of the proper conduct of their children's academic career as well as the appropriate management of their cognition, emotions, and behavior. The parents' lack of obedience, even implied, is interpreted as a significant factor in worsening the disabilities and in causing the potential defects to become activated.

The discourse on disabilities shapes and reflects the various models of responsibility, including parental responsibility and the division of labor between various social agents—private experts, state educational institutions, parents, and the disabled. The textual analysis reveals the fact that the parents, as those responsible for the failure of their children, are placed in positions that are often susceptible to supervision, evaluation, and criticism on the part of the experts. The ongoing sophistication of the experts in terms of therapeutic and pedagogical models creates an abundance of age-specific prescriptions from which the parents must learn rules of behavior and conversation with their children. The increase in the amount of rules, as well as the warnings about the development of malfunctioning if they are not abided by, creates a parent as someone who must monitor and locate signs of learning problems and neutralize them. If he does not do so, the signs are liable to show his guilt when the disability erupts. Normative parenting thus requires the development of habitual tendencies of continual concern for the child's success and practical steps to remove and neutralize any obstacles for achieving this along the way.

Seeking help from professionals is the recommended recourse for parents in coping with the array of negative feelings, such as helplessness, guilt, worry, and despair, which they may have at the beginning—the textual indicators of parental responsibility and caring.

Universalization and Empathization

The dominant experts of the burgeoning LD field stressed, from the very beginning, the great importance of their obligation to being sensitive—not only in reference to the care of the disabled child but also in reference to their various relationships with that child's parents and relatives. Narrativization of parental distress and suffering was a central motif in that expert literature. Indeed, this motif has been intertwined throughout the LD literature since the 1960s. Moreover, various kinds of textual mechanisms that were integrated into this narrativization of distress and suffering have contributed in total to produce an empathic climate in which readers of that literature are immersed.

Another central component which typified the attitude of the experts toward the parents was expressed in the textual structuring of the parent as an imaginary figure of a universal “every parent.” The qualities and conduct of this “every parent,” in relation to the disabled child, to other family members, to experts, and to the education system, were presented as natural, standard, expected, and required of every responsible and caring parent.

In a parallel and complementary fashion, this literature was characterized by the blurring and elimination of every social, class, and cultural characteristic of these parents. The legitimacy for this nullification of the context came from the reductionist biological models that were enlisted to clarify the pathologies under discussion. Therefore, it became trivial to draw parental figures as schematic and detached from any context, though they were signposted as empathic modifiers.

Problematic Parents of the Culturally Deprived

In contrast to the universal construction of the parents of the child with brain damage, and as opposed to the conventional rhetoric of empathization which was associated with this construction, in the cultural deprivation discourse, the parental figures were defined by means of particular ethnic, class, and cultural signifiers, using disempathic rhetoric. Rather than the “every parent” of the brain damage discourse, the parents of the culturally deprived were perceived as being able to operate in a limited number of social configurations—ethnic, class, and cultural ones. Hence, from the first, the discourse on cultural deprivation was characterized by particularization of the parental image which was its object. It also created a separation between the social groups they belonged to and groups identified as normative.

Parents of culturally deprived children were described as unfit in many ways (intellectually, motivationally, in terms of language, emotionally and socially) to help with advancing their children. The various researchers emphasized the dysfunctionality of the parents of culturally deprived children, as well as their inability to extricate themselves autonomously from this condition of dysfunctionality.³⁵ These parents of culturally deprived children were described as having negative qualities such as pessimism and defeatism.

Often, the psychopathologizing and disempathic description of the parents was outspoken: “The reason for this form of disturbance in the intelligence activity [of the culturally deprived child – O.K.] is to be found in the social and spiritual functioning of the environment. Here the reference is to insufficient motherly love, limited care-giving and above all the parents’ cultural inadequacy, poverty and primitiveness, which deprive them of the capacity to train, exercise and more importantly to expand effective social contacts,” explained Sachs (1971) adding that since “he cannot rely on anyone [in relation to the culturally deprived child – O.K.] he cannot be relied on either.”

The descriptions of the mothers and fathers were typically pathologized and there was sometimes even demonization of the family. Sachs (1971) describes the mothers in culturally deprived families thus: “These are schizoid depressive women.” He goes on to distinguish between several “typical mothering types” in these families. “The mother who is not yet mature enough for motherhood. She is not capable of devoting herself to the child while making concessions.” “The

³⁵ Explicit models of this type are that of Bettelheim (concerning autism) and of Ling (concerning schizophrenia). For a local list of culturally deprived parental stereotypes, see Sharni (1981). See also note 30 above.

ambivalent mother alternates between loving and rejecting the child,” “the disharmonic, unstable mother.”

The paternal profile is presented in clinical terms:

There is also the domineering father who works outside the home and thinks he deserves his quiet time and that dealing with children is the role of the mother only. Usually, it is the father who arouses fear: irritable, aggressive. His family bears the brunt of his disappointment at his failures at work and trembles before him (Sachs 1971).

He goes on to mention “[the father’s] rudeness, lack of control, manner of talking to the mother...excessive drinking, beating of the mother...”

Indeed, most of the descriptions of family and parents of the culturally deprived did not get to the point of demonization as described above, but they, nevertheless, were characterized by psychopathologization of the families and by disempathic descriptions of the parents. The weak father figure (Ezbe’oni 1984) and the distant mother figure (Eiger 1977) frequently presented the linkage between cultural deprivation and the pathological qualities of the parents in these families.

Hence, not only were the parents perceived as unqualified to help with solving the problem of lack of care but also in most cases they were perceived as exacerbating the problem and as the main cause of it. The parents were found not to be available for meaningful connections with their children and as being able to maintain only limited interaction with them (Ezbe’oni 1984). This kind of interaction would be likely to cause great harm in the formation of the self of the culturally deprived child, who was deprived of standard parental influence (Frankenstein 1977). The parents were presented as only sparingly reinforcing and supporting their children and thereby harming their proper development (Minkowitz 1969). “The mother of the average distressed family spends little time relating to the child and does not develop in him a feeling of closeness,” stressed Ashkenazi (1978). This absence of the feeling of closeness was expressed, in his view, in damaged development of thinking and perception of the environment. This position, whereby the problematic conduct of parents toward their children served as one of the major factors in the sub-standard cognitive development of the latter, was widespread among the experts on cultural deprivation. Thus, important experts like Minkowitz (1969), Frankenstein (1970), and Eiger and Kunik (1977), emphasized the destructive developmental repercussions for the children, that could be attributed to the relationship between parents and their children.

The professional literature emphasized two major series of causal links as responsible for the evolving of cultural deprivation: the one highlighted the cultural and socio-economic factors, and the other emphasized the clinical characteristics of the families. These two series were not mutually exclusive; they were usually employed concurrently and mutually as explanations for the evolving of cultural deprivation. The phrase coined by the experts, “poverty and primitiveness,” was used to discuss the diseased character of these two factors.

The non-standard development and the development of the symptomatology of cultural deprivation were perceived not only as being grounded in economic distress and cultural backwardness but also as a consequence of problematic, non-normative parental mediation. An example of this appears in Simon (1966) who relates to

parents as members of a sub-culture: “It is true that they are objectively needy, but they may also enjoy their regressive dependence and may be exaggerating their symptoms beyond what is necessary.” To the extent that the parents do this, they are not fulfilling their normative educational role, but, through their actions, they are expressing “that pedagogical suicide arising from their artificial infantilization.”

We have found that, in the discourse on cultural deprivation in relation to the parents, there are particular outlines of the ethnic origins and class as well as the (negative) character of socialization in their family. This duality allows the experts to assert, on the one hand, that cultural deprivation is a product of cultural and socio-economic factors which helped to create, preserve, and reproduce the identification between cultural deprivation and Mizrahi origin and low class (thus assisting in the creation and preservation of the establishment’s distinction between these populations and modern Western civilization that is associated with the establishment).

On the other hand, the concrete discussions of the negative qualities of the parents as the missing link which was critical in activating the cultural deprivation syndrome enabled the experts to focus the blame on parents for this development. As the parents are constructed and understood as being to blame for the faltering of their children, the experts and the establishment are likely to rid themselves of responsibility for realistic policy to remedy the gaps and social distress. In any event, for our purposes, the textual construction of the parents of the culturally deprived as being responsible and to blame for the cognitive, motivational, emotional, and behavioral behavior of the children, is important. This is despite the fact that the discourse on cultural deprivation highlights contextual categories of class and ethnic origin—explanatory categories which seem to negate naïve perceptions about blame and the failure or lagging of their children.

This formation of parental blame, which was established by relating to the categories of poverty and culture, is expressed by Eiger (basing herself on the theory of Frankenstein):

According to Frankenstein, an orientation is created among poor people about their deficiency as well as what they might have one day (“the haveable”) which prevents the individual from adopting an autonomous attitude to the world. The relationship to the immediate and the real reinforces in the individual the fact that he is a thing subject to the randomness and arbitrariness of external conditions; as a result, he does not get to the point of confrontation and coping with reality, events, and the people in his world. This is the source of the impersonal relationship of the poverty-stricken parents toward their child. These parents treat the child like an object in terms of fulfilling their role toward him and in regard to the expectations they have of him [Emphasis added]

Furthermore, she claims that “in primitive cultures, the factor that weakens the personal relationship on the part of the parents is the absolute authority of tradition. Life in a closed system of beliefs, unequivocal values and fixed behavior patterns limits the opportunities for the individual self to develop and in its stead there develops a collective self in connection with which the individual feels himself to be an object.”

According to Eiger, parents of culturally deprived children shaped the weak, passive self among their children: “The correlation between the weakness of the self and the culture of poverty and primitive cultures is a consequence of the quality of

the parents' attitude to the child" (1975). In general, the various studies allow us to outline a very problematic profile of a parent of a culturally deprived child. This profile places the latter in a negative light, to say the least—the characteristics associated with him in the literature include cognitive characteristics (obstinacy, lack of logic, lack of education, ignorance, and low achievement); personal characteristics (lack of belief in his own strength, passivity, low frustration threshold, and insecurity); family characteristics (lack of understanding of family needs, lack of emotional relationship with the children and spouse, authoritarianism on the part of the father, and submissiveness on the part of the mother); and other characteristics (cultural backwardness, reduced connection with the surrounding culture, and dependence on external authority).

Attempts to minimize the implications of the pathologization of the portrayal do nothing to blur the psychopathologization. An example is Eiger's remark that the impersonal attitude to the child does not mean that the parents do not express love, affection, or concern for their children, but because of the weak self of the parents, the expression of emotion is impulsive, uncontrolled, and lacking in constancy which is the reason for the rapid transitions from love to rage, from neglect to devotion, etc. This attitude does not allow the child to feel the constancy of the parents' attitude and so he is not able to internalize it (Eiger 1975).

The parent of a culturally deprived child is identified, on the one hand, with Mizrahi ethnic origin and lower class, and, on the other hand, his self is stigmatized through a rich index of negative psychological indicators. The words poverty and Mizrahi-ness were used in written works as indications of backward cultures.

Juxtaposition Between Backward Parents and Normative Parents: Two Cultures of Blame

Another rhetorical mechanism, which emphasized, even more, the problematic qualities of parents of culturally deprived children, was the textual juxtaposition of the backward types of parents with the normative types of parents—a juxtaposition which aided in their mutually opposing formation. Thus, for example, under the sub-section on parents in a faltering population, Avni (1978) claimed that parents of students from the established–developed population³⁶ are usually a positive and stimulating factor in the education of their children and encourage their studies. They also know how to assess correctly the importance of proper relations between the school and the parents; hence, they themselves initiate activities to strengthen this connection by visiting the school, the principal, the teacher, or the nurse through constructive activity on the parents' committee, etc. Conversely, the student being raised in a deprived environment does not always benefit from scholastic encouragement and stimulation from his parents. At times he encounters apathy, and at times his parents deny him regular learning conditions through over-using him in services and chores in the home.

This is a distorted reflection in which the parents are presented as backward and not fulfilling their parental and pedagogic role.

³⁶ Mainly of an Ashkenazi European origin.

As stated, an explicit juxtaposition between the two types of parents (as well as between the two types of psychopathologies) in the literature is rare. To a large extent, the Mizrahi parents were identified as being to blame for their children's backwardness. It is paradoxical that these parents were found to be responsible for their children's failure because the cultural deprivation discourse itself emphasized the class and cultural marginalization of these parents in the social structure and blurred the socio-critical potential by its very presence and emphasis on this structural marginalization.

In well-known senses, the discourse on cultural deprivation is antithetical to that of brain damage and, later, to that of LDs. The latter proposed a different special education taxonomy. They laid emphasis on the biological elements in the act of explicating learning and behavioral difficulties among children. As mentioned, they also emphasized the importance of expert sensitivity for the parents' distress and asserted that there was no parental blame or responsibility in the engendering of disabilities.³⁷

The two discourses exemplify two distinct types of parental cultures of blame and responsibility: The LDs discourse reflects an inclusive and integrative model of blame and responsibility. The parents are not causally blamed for engendering their child's disability, but still they are responsible for the management of that disability, whereas the cultural deprivation discourse reflects an exclusive model of blame and responsibility—the parents are causally blamed for engendering their child's disability, while not being responsible for the management of that disability. Both of the discourses assign a central but different role to the parents due to the parents' important position as mediators between the public spheres of the medical and educational discourse and the private sphere of the family.³⁸

On the Obliteration of the Ethnic and Class Context in the LDs Discourse

The references of the experts' texts within the LDs discourse to the emotional world refer also purportedly to every parent. The emotional signifiers' index outlines, seemingly, the deserving figure of the universal "every parent." However, it is, in effect, based on the emotional formation (and its practical derivatives) of a social group distinguished by class, culture, and language,³⁹ that social class that

³⁷ To date, there has been no expression of any correspondence between these two special education discourses. The author of this study wishes to fill in this gap in an article which he is about to publish. For now, it is sufficient to note that explicit correspondence between the two is almost non-existent. The cliques of education experts have organized themselves in such a way that two structured discourses have been created—one of experts in cultural deprivation and the other on learning disabilities. Although experts from both groups have published their studies in the same journals, in general almost no attempt has been made to integrate the two discourses, not to mention to consider the meaning and sources of the separation between the discourses, both of which have dealt with educational problems having similar characteristics.

³⁸ More about the parental culture of blame and the central role of parents in mediating between the two spheres can be found in Singh (2004).

³⁹ The indicators of being well-established, culturally nurtured, using standard language, and possessing economic resources hint at the ethnic identity of many of these parents as being Ashkenazi (or at least not Mizrahi).

habitually associates the description of “transparent” parents with specific social characteristics. Thus, normative parenting is a moral imperative as well as the possible privilege of a distinct group of parents, a potential group of readers on the imaginary horizon of expert literature—a group that can be located within class, cultural, and language boundaries despite its being composed of a collection of individuals. The good parent is portrayed by means of a series of cognitive, behavioral, emotional, and personal characteristics. This parent is designated as intellectually, motivationally, emotionally, and socially capable of understanding the child’s difficulties, able to learn the professional language of disabilities experts and even to cooperate with them in the act of education. In the descriptions of and references to parenting found in the literature, a picture can be drawn of its characteristics as natural, as it were, as literate (i.e., in command of the high registers of language, written and spoken), as cultured (i.e., as possessing a rich collection of knowledge and having the intention of investing in academic enrichment), as intelligent, ambitious (like his child as oriented to a successful life) and as one who conducts an enriched lifestyle (cinema, museums, leisure-time conversations, and other similar practices) all of which have therapeutic significance. This parent is even characterized as possessing a series of emotions and sensitivities that enable him to aid in the empowerment of his child vis-à-vis the education establishment and also to enlist the assistance of experts without being rejected or negated by them. The absolute majority of the writers tended to blur and obliterate class, social and cultural characteristics of the families they described. This blurring and obliteration were aided by the ethics of the discipline that obliged anonymity for those mentioned in the various books, publications, and reports. This code of ethics makes it difficult to locate in the writings direct and explicit attribution of class and social affiliation of the families mentioned. In this way, it assists in the encryption of the social stratification and cultural classification of the studies conducted and reported in the research and professional literature. The class and cultural boundaries of the objects written about can be explicitly uncovered only from laconic and incidental remarks related to the profession, to place of residence or to leisure-time culture of one family or another. For example, Segal (1969), in referring to groups of parents of the learning disabled and to their capabilities, he emphasizes the importance of pressure groups on educational decision makers and the strength of their potential influence by making an incidental comment: “In every group of parent – as in every group of people – one can find a broad spectrum of professions: lawyers, doctors, writers, and publicists.” From writings like these, a picture is drawn of the “core consumers” of the disabilities discourse in the Israeli context, as referring to established and educated groups (historically Ashkenazi and well-established), who are able to derive social benefit from the services of experts in LDs. However, in most of the written works, these social contours were completely blurred and were replaced by individual characteristics. This substitution facilitated the identification of parents from special social groups as potential “natural” partners of the experts without there being any explicit mention in the discourse of their social characteristics. In this manner, the experts succeeded in a textual construction of the symbolization of universal unity (the presentation of “every parent”) and at the same time in

confining their expertise to those socially distinguishable parents whose so-called individual qualities were valid for them.

Based on these qualities, in effect, established, educated parents are identified in the expert literature, as potential, natural partners, not only in individual therapy for the disabled children but also in studying and dissemination the body of knowledge about LD itself. Not every parent was able to be a partner for the experts in therapy, consultation, learning, and dissemination of the discourse. In order to undertake this, he was required to adopt and develop characteristics of an “every parent”. However, not every parent was found to be suitable for this, culturally, economically, intellectually, and even morally. This figure could serve as a role model only for those established and educated social groups in whose image it was shaped in the first place. In this way, a justification was provided for the expert discourse to create conceptual, professional, and institutional distinctions between the objects of therapy relevant to experts on disabilities and those to whom other therapeutic logics ought to apply. This appearance of discursive, conceptual distinctions, and professional and institutional division of labor—valid for social, ethnic, and class distinctions—serves as the historical case study of the discourse on cultural deprivation in Israel.

Summary

In summary, it seems that under the aegis of the two discourses, both the cultural deprivation and the LDs ones, two special educational tracks developed in Israel. The two tracks defined differentially the likelihood of their distinct populations to succeed or, conversely, to fail.⁴⁰ The two tracks formed and organized around different representations of the “self” that were attributed to the populations that served as their subjects of study: The cultural deprivation track was organized around a contaminated self (cognitively, morally and behaviorally). The brain-damaged track (eventually the LDs track) was organized around a rehabilitated self in these respects.

The logics (*raison d'être*) of the two tracks can be explained in terms of contrasting educational ideas. The logic of the cultural deprivation track can be characterized as an evolutionary idea of progress (the children are to be advanced as far as possible, although the disabilities are not totally reparable). The logic of the LD track can be characterized as a mechanistic idea of correction (the disabilities or rather the disabilities' manifestations are reparable).

That is, the cultural deprivation discourse reflected and expressed evolutionary pedagogical trends by which the support to be given to culturally deprived children was of great importance, even though it was to be expected that their level of

⁴⁰ Procedures of exclusion, segregation, and institutional tracking along ethnic lines of Mizrahi Jewish children elicited hardly any opposition, as they were taking place under the banner of “tipuach” (the Israeli “nurture” version of the Cultural Deprivation discourse). For example, a considerable number of Mizrahi children have been placed in vocational schools by this procedure (Mizrachi 2004; Swirski 1990; Shavit 1990) and referred to special education programs (Tsur 1991). According to these authors, these processes have played a central role in shaping the current stratified social makeup of Israeli society.

development (educationally speaking—their abilities, capacities, and as a result their achievements) would be limited. The reason for that limitation stems from the cultural, socio-economic, and familial influences that shaped those children from their infancy. These influences were assumed to have made an impression on their self and would not be totally repairable (or reversible).

As opposed to that, the LDs discourse reflected and expressed corrective pedagogical trends by which suitable instruction and care can fix the disabilities or at least neutralize their problematic manifestations. Therefore, there was in that case an expectation that despite the existence of the disabilities, proper instruction could remove any limitation on the child's future achievements (the severity of the disability notwithstanding). And in regard to the learning disabled children's abilities and capacities, these children purportedly had no anomalous cultural, socio-economic, or familial history. Therefore, an adequate enough mechanistic intervention could uncover those abilities and capacities (without regard to their degree of severity).

The progress idea was embedded in a phylogenetic perception of human development. In the cultural deprivation discourse, it was not possible to separate the child's defective self from the problematic environmental circles which surrounded it. That child's difficulties (as well as their origins) were not his alone but were rather part of a broad backward population. The danger immanent in such a problematic child is not manifested only (or mainly) in his future failures but also in the negative influences of these failures on society at large. That being the case, the treatment methods had to provide an answer to a collective problem, not just to an individual problem of this or that child. Moreover, since the problem was perceived as collective, it was not an option to find an overall solution to the problem of the individual child. Only a structural and systematic treatment of the backward populations—a treatment whose aim was to ameliorate them over time—could eventually be manifested in the form of a child having an unblemished self.

The correction notion was embedded in an ontogenetic perception of human development. The child's defective self was examined in light of psychological theories and developmental models that focused on the developmental phases of that particular human individual from birth, on difficulties and deviations from the normal stages, and on adequate treatment methods. That child's difficulties (as well as their origins) were self-contained, and hence it was possible to locate, isolate and “surgically” fix them. Because of that, the aim of the treatment was first to advance the child's development (in the ontogenetic-particular sense) as an individual, to speed up that development, or to remove the barriers placed by the disabilities before that development.

The phylogenetic understanding of the difficulties of the culturally deprived was manifested, therefore, in identifying these difficulties as qualitatively different from similar (and also from seemingly identical) difficulties of children who did not belong to the culturally deprived population (i.e., cultured or normal children as conceived by the experts). So, culturally deprived children were socially constructed as essentially different from other children.

The ontogenetic understanding of the difficulties of the learning disabled was manifested, on the other hand, in identifying these difficulties as mere statistical variants on a mainly normative developmental continuum. It is true that at times

there could be found among some learning disabled children severe deviations from or significant delays in the expected normal development. But still, even in these severe cases, the children's difficulties were identified as only quantitatively different from the difficulties of non-disabled children. The difficulties also were conceptualized as only technical and not as essential. Thus, learning disabled children oxymoronically were socially constructed as normal children in all senses even though they suffered from disabilities.

The two special educational tracks, the two self-images that were formed within their framework, as well as the diverse educational ideas and the different development conceptualizations that characterized them, reflected historically the unequal and hierarchical ethno-class social structure of Israeli society, at the same time that they were contributing to its social reproduction.

The expectations and recommendations of the experts should be viewed in the broader educational and social context—during the 1960s, a segregationist educational policy was engendered in Israel; a policy which has become the prevailing one (Ziv 1980). Since the establishment of the State of Israel in 1948 and up to the 1960s, the educational ideology of equality in education was dominant. The meaning of this was that, theoretically at least, there were equal conditions and equal curricula in all of the schools (Ziv 1980). Many have written about the formation of the segregationist educational ideology during the 1960s focusing mainly on the now discredited discourse of cultural deprivation. That ideology developed against the background of the incoming waves of immigration to Israel from states which were conceived as backward, the negative perceptions of many of the immigrants on the part of the indigenous population, and the widespread local public belief in the validity of the I.Q. test.⁴¹

The simultaneous existence of the LDs discourse (a discourse that emerged in Israel in those very years) highlights an otherwise hidden aspect in the historical formation of the aforementioned segregationist ideology: The currently rejected widely critiqued cultural deprivation category did not emerge in an educational vacuum devoid of alternative patho-educational conceptualizations. It was the brain-damaged category (that eventually became the LDs category) which emerged simultaneously serving as a contrasting category to it. Subjects divided historically by ethnicity, and class were reflected and produced in this alternative LDs category. It seems that this special education track of LDs could have been to a large extent the historical parallel of the cultural deprivation track. That is to say that the LDs track tended to serve mainly the Ashkenazi, well-established, educated, and well-off population.⁴² It seems that the LDs track was used as a special education alternative for the strong parents.

⁴¹ These trends are expanded further in Swirsky (1988).

⁴² This historical parallel can be understood from the comparison of the category of learning disabilities and that of cultural deprivation in this article as well as from the textual structuring of the target audiences of disability experts in my forthcoming article.

Conclusion: Differential Expectations

Distinct etiological attributions as well as differential interpretations of identical symptomatology reinforced the experts' prognostic expectations and practical and policy recommendations regarding how the children with difficulties should be dealt with and also reflected those expectations and recommendations. The critical literature is full of descriptions of the low expectations of the educational experts of the culturally deprived children. These low expectations are associated in that literature with the term "the Pygmalion effect" which negatively contributed to the defective molding of these children in the image of their widespread negative imageries. In accordance with this, the destructive policy consequences suited to these experts' positions regarding the culturally deprived population were extensively discussed in written texts.

"The question is raised [remarked Darin as early as 1971- O.K.] by many specialists in remedial teaching: what is the difference between the remedial teaching of language, reading, writing and arithmetic difficulties originating in abnormalities of the functioning of the brain, and the remedial teaching of learning difficulties originating in an environmental background?" And, in the same vein, we can broaden the question and consider whether there were any differences between the expectations of experts of culturally deprived children and their expectations of learning disabled children? If indeed, there were such differences we should ponder how, they manifested themselves. Moreover, attention should be paid to the differences in the recommendations for methods of care and assistance.

In retrospect, it is surprising to realize what a large difference there was between the expectations of the disabilities experts regarding the children who had difficulties assessed as being based on an organic background and their expectations regarding the culturally deprived children. More often than not, the difficulties described were the same difficulties, the failures were the same failures, and the clinical picture was also identical in both cases. However, while concerning the culturally deprived children, the disabilities experts adopted the customary and accepted motifs "failure expected in advance" and "the need to lower expectations," concerning the learning disabled children they adopted different motifs. Concerning these LD children, they developed a positive stance aided by idioms, motifs, and argumentations that were embedded in the semantic field of success. It appears that the boundaries of the individual potential were understood, conceptualized, and constructed very differently in the two cases.

While the first special education track served many children as an exclusive route that relegated them to the margins of society, the second special educational track served other children in a different way, seemingly (even only potentially) as a supportive, inclusive, and integrative route. Whereas the educational and social manifestations of the first track were extensively examined and studied, the educational and social manifestations of the second track still wait for research and examination.

References

- Adar, L.
1968 *The Motivation for Learning and the Student's Personality*. Jerusalem: The Education Department of the Hebrew University and the Ministry of Education and Culture.
- Adiel, S.
1969 *The Slow Class in the Secondary School*. Jerusalem: The Central Committee for the Implementation of the Reform, The Ministry of Education and Culture.
- Arazi, S.
1972 *Learning Disabilities among Brain Damaged Children—A Clinical Diagnosis*. *Child Development* 1(3): 10–23.
- Arieli, M, Y Kashti, and S. S. Shalsky, eds.
1997 *The Education and Instruction Lexicon*. Tel Aviv: Ramot.
- Ashkenazi, D.
1978 *Verbal Classification Ability of Terms in Various Content Worlds among Culturally Deprived as well as Well-Off Students*. M.A. Thesis, University of Tel Aviv.
- Avni, D.
1978 *'Nezah Israel' School: The First Special School in Israel*. M.A. Thesis, Tel Aviv University.
- Birch, G.
1970 *The Brain Damage Problem in Children*. *Child Development* 1(1): 3–11.
- Belgor, R.
1977 *The Psychology of Reading: The Diagnosis of its Dysfunctions and their Remedial Methods*. Jerusalem: Schocken.
- Ben-Eliahu, S.
1976 *Inclusion: A Guide for Education Workers in Primary Schools; Explanation and Proposed Action Programs for the Care of Culturally Deprived Students in the Academic Year 1977*. Jerusalem: Ministry of Education and Culture.
- Berger, PL, and T Luckman
1967 *The Social Construction of Reality: A Treatise in the Sociology of Knowledge*. London: Penguin Press.
- Bettelheim, B.
1967 *The Empty Fortress: Infantile Autism and the Birth of the Self*. New York: Free Press.
- Bury, MR
1986 *Social Constructionism and the Development of Medical Sociology*. *Sociology of Health and Illness* 8(2): 137–169.
- Fairclough, N.
1992 *Language and Power*. London: Longman.
- Carter, A, W Hall, and J Illes, eds.
2012 *Addiction Neuroethics: The Ethics of Addiction Neuroscience Research and Treatment*. London: Academic Press.
- Chasin, Y.
1975 *The Culturally Deprived Student—Victim of Labeling*. *Megamot* 21: 140–156.
- Clarke, A.E., J. K. Shim, S. Shostak, and A. Nelson
2009 *Biomedicalising Health, Diseases and Identities*. In *Handbook of Genetics and Society: Mapping the New Genomic Era*. P. Akinson, P. Glasner, and M. Lock, eds., pp. 21–40. London: Routledge.
- Clarke, AE, L Mamo, JR Fosket, JR Fishman, and JK Shim, eds.
2010 *Biomedicalization: Technoscience, Health, and Illness in the U.S*. London: Duke University Press.
- Colyar, J., and K. Holley
2010 *Narrative Theory and the Construction of Qualitative Texts*. In *New Approaches to Qualitative Research: Wisdom and Uncertainty*. M Savin-Baden and C. H. Major, eds., pp. 70–79. London: Routledge.

- Conrad, P
 1992 Medicalization and Social Control. *Annual Review of Sociology* 18: 209–232.
 2006 Up, Down, and Sideways. *Society* 43(6): 19–20.
 2007 *The Medicalization of Society: On the Transformation of Human Conditions into Treatable Disorders*. Baltimore: Johns Hopkins University Press.
- Doron, R
 1970 Similarity and Difference between ‘Therapeutic Classes’ and ‘Remedial Instruction Classes’. *Studies, Perusals and Recommendations Series B* 102–130.
- Daniel, S
 1971 The Culturally Deprived Student. *Education* 309–312.
- Darin, A
 1971 Learning Problems from a Neurological Perspective—the Contribution of Medicine to Clarification of Problems of Language, Reading and Writing. *Education* 116–123.
 1976 Delay in Maturation and Dysfunctions in Brain Functioning. *In* *Learning-Disability: Characteristics, Diagnosis*. R Kidron, ed., pp. 8–18. Ministry of Education and Culture: Treatment.
 1984 *Neuropsychology and the Deviant Child*. Haifa: Oranim.
- Eiger, H
 1975 *Rehabilitative Instruction for Culturally Deprived Students*. Tel Aviv: Sifriat Hapoalim.
- Eiger, H., and N. Kunik
 1977 *Teacher Training for the Rehabilitation of Culturally Deprived Students’ Thinking Processes*. Iyunim Behinuh 23–38.
- Eilon, A
 1970 Emotional Delays in the Learning of Reading. *Child Development* 1(1): 28–37.
- Emerant, S
 1968 Learning Difficulties and their Psychological Significance. *Education* 1: 193–199.
- Ezbe’oni, Z
 1984 *The Influence of Learning the Average Concept using the Method of Guided Individualized Instruction, Using Findings from Developmental Psychology*. Tel Aviv: On the Achievements of Culturally Deprived Adolescents.
- Frankenstein, C
 1969 *Youth on the Margins of Society: the Forms of Youth Delinquency*. Tel Aviv: Ahiasaf.
 1970 *Rehabilitation of the Injured Intelligence*. Jerusalem: School of Education of the Hebrew University and the Ministry of Education and Culture.
 1977 *Sincerity and Equality: Afterthoughts of a Psychologist and an Educator*. Tel Aviv: Sifriat Hapoalim.
 1981 *They are Rethinking: Chapters in the Remedial Instruction Doctrine*. Tel Aviv: Am Oved.
- Gaines, AD
 1992 From DSM-I TO III-R; Voices of Self, Mastery and Other: A Cultural Constructivist Reading of U.S. Psychiatry Classification. *Social Science and Medicine* 35(1): 3–24.
- Goffman, E
 1983 *Stigma*. Tel Aviv: Reshafim.
- Hamdani, M.
 2003 *The educational programs in special-education in the years 1954-1997: An historical documentation of special-education in Israel*. Ministry of education.
- Hartwell, C. E.
 1996 The Schizophrenogenic Mother Concept in American Psychiatry. *Psychiatry: Interpersonal and Biological Processes* 59(3): 274–297.
- Hess, I
 1966 The Integrated Approach to the Child who Suffers from Dysfunctioning of the Brain. *Megamot* 14(4): 335–346.
- Horsdal, M
 2012 *Telling Lives: Exploring Dimensions of Narratives*. London: Routledge.
- Kashti, Y
 2009 *Towards a Multicultural Education in Israel*. *Alpayim* 34: 261–280.
- Katchergin, O
 2009 *Learning (in) Disorder: A Sociological Perspective on Learning Disabilities in Israel*. Ph.D. dissertation, Tel Aviv University.

- Kidron, R
1985 *Difficulties in Learning Arithmetic: Characteristics, Diagnosis and Treatment*. Tel Aviv: Otzar Hamoreh.
- Linton, S
1998 *Claiming Disability: Knowledge and Identity*. New York: New York University Press.
- Margalit, M
1984 *In Constant Movement: The Hyperactive Child at Home and at School: Educational and Psychological Aspects*. Tel Aviv: Sifriat Hapoalim.
- Minkowitz, A
1969 *The Culturally Deprived Student: Problems of Diagnosis, Etiology and Rehabilitation*. Jerusalem: School of Education of the Hebrew University and the Ministry of Education and Culture.
- Mizrachi, N
2004 From Badness to Sickness: The Role of Ethnopsychology in Shaping Ethnic Hierarchies in Israel. *Social Identities* 10(2): 219–243.
- Moghim, Y
2012 Anthropological Discourses on the Globalization of Posttraumatic Stress Disorder (PTSD) in Post-Conflict Societies. *Journal of Psychiatric Practice* 18(1): 29–37.
- Nissan, M
1973 *The Preparatory Program of Academic Studies for Soldiers with a Cultural Deprivation Background: Description, Evaluation and Analysis Attempt—Final Report*. Jerusalem: School of Education, The Hebrew University.
- Nye, RA
2003 The Evolution of the Concept of Medicalization in the Late Twentieth Century. *Journal of History of the Behavioral Sciences* 39(2): 115–129.
- Oliver, MJ
1990 *The Politics of Disablement: A Sociological Approach*. Basingstoke: Macmillan.
- Ortar, G
1966 On the Possibility of Preventing Cultural Retardation among Infants. *Megamot* 14: 78–86.
- Peres, Y
1976 *Ethnic Relations in Israel*. Tel Aviv: Hapoalim.
- Petrina, S
2006 The Medicalization of Education: A Historiographic Synthesis. *History of Education Quarterly* 46(4): 503–531.
- Pulkingham, J, and S Fuller
2012 From Parent to Patient: The Medicalization of Lone Motherhood through Welfare Reform. *Social Politics* 19(2): 243–268.
- Razi, T
2006 *The Construction of the Nation's Margins: Neglected Youth in Tel-Aviv in the Thirties and the Forties*. Ph.D. dissertation, Tel Aviv University.
- Riessman, CK
2008 *Narrative Methods for the Human Sciences*. Los Angeles: Sage Publications.
- Riska, E
2003 Gendering the Medicalization Thesis. In *Gender Perspectives on Health and Medicine*, Vol. 7. MT Segal, V Demos, and J Jacobs Kronenfeld, eds., pp. 59–87. Boston: JAI.
- Robinson, HA
1966 Reading and the Reading-Deprived Child. *Education* 177–194.
- Roswell, F, and G Natchez
1969 *Reading disability: Diagnosis and Treatment*. Tel Aviv: Otzar Hamoreh.
- Sachs, S
1971 On Education Difficulties as a Challenge for the Educator. *Education—Pedagogy and Psychology Quarterly* 302–308.
- Shalgi, R.
1987 On Teaching Literature to Culturally Deprived Students. *Issues in Education* 96.
- Sharan, S, and Y Sharan
1969 *Learning Disabilities and Their Correction*. Tel Aviv: Sifriat Hapoalim.

- Sharni, S
1981 Culturally Deprived Fathers and Mothers in the View of Professionals. *Megamot* 27(2): 190–200.
- Shavit, Y
1990 Segregation, Tracking, and the Educational Attainment of Minorities: Arabs and Oriental Jews in Israel. *American Sociological Review* 55(1): 115–126.
- Shitrit, SSh
2004 *The Mizrahi Struggle in Israel 1948–2003*. Tel Aviv: Am Oved.
- Shmilowitz, M.
1984a Alternative Instruction for possible potential extraction according to Karl Frankenstein. Tel Aviv: Ministry of education, Mofet institute.
- Shmilowitz, M.
1984b Remedial Instruction—Its Contribution to Teacher Training. *Processes in Education, Instruction and Teacher Training* 203–208.
- Siegel, E
1969 *Helping the Brain Injured Child: A Guide for Parents*. Tel Aviv: Nitzan.
- Simon, EA
1966 On the Educational Doctrine of Karl Frankenstein. *Megamot* 14: 7–54.
- Simpson, S
1974 *A guide for the teacher of the brain-damaged child*. Ramat Gan: Bar-Ilan University.
- Simpson, S
1985 *Brain Damage and Learning Disabilities*. Tel Aviv: Ramot.
- Singh, I
2004 Doing their Jobs: Mothering with Ritalin in a Culture of Mother-Blame. *Social Science and Medicine* 59: 1193–1205.
- Smilansky, S
1978 Bringing Forward Reading Instruction for Culturally Deprived Children. *Primary Education* 22: 5–12.
- Stahl, A
1977 *Language and Thought of Culturally Deprived Students in Israel*. Tel Aviv: Otzar Hamoreh.
- Swirski, S
1988 I.Q. in Israel. In *The I.Q. myth*. G Bloom, ed., pp. 154–162. Tel Aviv: Mifras Books.
1990 *Education in Israel: Schooling for inequality*. Tel Aviv: J & B Thompson.
- Thompson, J. B.
1984 *Studies in the Theory of Ideology*. Cambridge: Polity.
- Toolan, MJ
2001 *Narrative: A Critical Linguistic Introduction*. London: Routledge.
- Tsur, A
1991 *Mental Health Discourse in Israel as a Product and Producer of Power/Knowledge*. PhD Thesis Submitted to the Hebrew University, Jerusalem.
- Zalashik, R
2008 *History of Psychiatry in Palestine and Israel, 1892–1960*. Tel Aviv: Hakibbutz Hameuchad.
- Ziv, A
1980 *Psychology in Education*. Ramat Aviv: The Open University.
- Zola, I
1978 Medicine as an Institution of Social Control. *Sociological Review* 20: 487–504.